

Martinez Alejandro, Rosa

MRN: 110710532

Office Visit 7/18/2024
PAV WH Multidisciplinary
Oncology Clinic

Provider: Pruitt, Aaron M, MD (Hematology and Oncology)
Primary diagnosis: Malignant neoplasm of sigmoid colon (CMS/HCC)
Reason for Visit: Follow-up; Referred by Patel, Reema A, MD

Progress Notes

Morgan, Rachael M, PharmD (Pharmacist) • Pharmacy

Pharmacy Hematology/Oncology Treatment Note

Rosa Martinez Alejandro is a 30 y.o. female with metastatic colon cancer

[Cancer Staging](#)

No matching staging information was found for the patient.

Study Patient: no

Treatment Plan reviewed for FOLFIRI +Panitumumab x2 q14d every 28 days.

- ☒ Follow-Up Clinical Review for Cycle 4 Day 1,15 (with Panitumumab added)
- ☐ Follow-Up Clinical Review for Continuous Oral Therapy

Interval History: Ms. Martinez Alejandro did not tolerate FOLFOX and only received 1 dose. Symptoms of tremors, limp stiffness, difficulty sleeping, fever/night sweats, etc and port infection. Based on review of literature, possibly related to oxaliplatin so changed to FOLFIRI. Will continue without bolus 5FU/LCV. KRAS mt neg so added panitumumab.

Today's Wt:

Wt Readings from Last 1 Encounters:

07/18/24 35.4 kg (78 lb 0.7 oz)

Dosing Wt: 36.2 kg
Dosing Ht: 150.9 cm
DosingBSA: 1.25 m2

Recent Labs:

Lab Results

Component	Value	Date
WBC	2.99 (L)	07/18/2024
HGB	11.8	07/18/2024
HCT	35.3	07/18/2024
MCV	92	07/18/2024
PLT	326	07/18/2024

Lab Results

Component	Value	Date
GLUCOSE	82	07/18/2024
CALCIUM	9.1	07/18/2024
NA	137	07/18/2024

K	3.4 (L)	07/18/2024
CO2	23	07/18/2024
CL	103	07/18/2024
BUN	10	07/18/2024
CREATININE	0.40 (L)	07/18/2024

Lab Results

Component	Value	Date
ALT	18	07/18/2024
AST	21	07/18/2024
ALKPHOS	96	07/18/2024
BILITOT	<0.2 (L)	07/18/2024

Lab Results

Component	Value	Date
NEUTROABS	1.84	07/18/2024

Lab Results

Component	Value	Date
MG	1.9	07/18/2024

No results found for: "TSH"

Lab Results

Component	Value	Date
URINEPRO	Negative	03/14/2024

Vitals:

Visit Vitals

BP	94/58
Pulse	79
Temp	36.7 °C (98.1 °F) (Oral)

Other Relevant Monitoring:

Treatment/Therapy Plan:

Panitumumab 6mg/kg (200mg) IV D1, D15
5-FU 2400mg/m2 (3,000mg) IV D1, D15 over 46h
Irinotecan 180mg/m2 (224mg) IV D1, D15
Every 28 days

☒ 5-FU bolus and LCV dropped for predicted tolerability

Current Treatment Plan History:

FOLFIRI
C1: 3/28, 4/11/24
FOLFIRI+Panit

C1: 4/25, 5/9/24
C2: 5/23, 6/6/24
C3: 6/20, 7/5/24
C4: 7/18, 8/1/24

Prior Treatment History:

FOLFOX
C1: 2/26/24

Plan:

Patient will return to clinic in 4 weeks. Will follow-up at that time.

Pharmacist Attestation: Rachael M Morgan, PharmD 7/18/2024 10:16 AM

Progress Notes

Pruitt, Aaron M, MD (Fellow) • Oncology

Cosigned by: Patel, Reema A, MD at 7/18/2024 5:18 PM

Attestation signed by Patel, Reema A, MD at 7/18/2024 5:18 PM

I saw and evaluated the patient with the resident/fellow. I discussed the case with the resident/fellow and agree with the findings and plan as documented.

Labs reviewed with respect to bone marrow, renal, and hepatic function and appropriate to proceed with treatment.

Orders reviewed and signed.

Rising tumor markers, will have scans at next visit, signatera drawn today.

45 minutes were spent with the patient of which 30 or more were counseling minutes regarding plan and coordination of care and follow up lab and/or scan review, scheduling, symptom management, and possible treatment side effects of anti-cancer therapy.

General Medical Oncology Clinic Note:

[Cancer Staging](#)
Malignant neoplasm of colon (CMS/HCC)
Staging form: Colon and Rectum, AJCC 8th Edition
- Clinical stage from 7/18/2024: Stage IVC (cTX, cNX, pM1c) - Unsigned

Treatment Plans

Name	Type	Plan Dates	Plan Provider
Active FOLFIRI + Panitumumab: Leucovorin + Irinotecan + Fluorouracil + Panitumumab **PAP	Oncology Treatment	4/24/2024 - Present	Reema A Patel, MD

Name	Type	Plan Dates	Plan Provider
Replacement** Every 14 Days x 2 Every 28 Days			

Subjective

Interval history:

No particular complaints today. She states that overall she was feeling better than prior to starting treatment. She was hoping to begin working who did a leak again soon. No infectious complications, fevers, rigors with his last cycle. She continues to have a mild acneiform rash on the face but takes her doxycycline as prescribed and says this has helped overall. This visit was conducted in its entirety with the help of an in person spanish interpreter.

Oncology History Overview Note

ID: metastatic colon cancer with ovarian mets, encasing local vasculature, peritoneal disease, MSI stable, MMR proficient

NGS - ATM, SMAD4, TP53, NRAS/BRAF WT (KRAS copy gain), TMB 1.1 m/MB

- p/w lower abdominal pain x 1 month associated w/ n/v and 11 pounds of weight loss
- 1/12/24 - CT abdomen/pelvis revealed circumferential wall thickening involving the sigmoid colon with pericolonic nodules. Also noted to have enlargement of the right greater than left ovaries worrisome for metastatic disease and confluent paraaortic lymphadenopathy in the lower abdomen
- 1/18/24 - established with Dr. Bhakta and underwent a flex/sig revealing a single friable, fungating, invasive and malignant appearing mass which was not traversable, in the distal sigmoid colon. CEA 3.0, CA 19.9 - 2,004, CA 125 - 92. Pathology c/w poorly differentiated carcinoma.
- 1/18/24 - repeat CT CAP showed no evidence of disease in the chest. Long segment of thickening of the proximal sigmoid colon with adjacent mesenteric mass and soft tissue deposits encasing the aortic bifurcation and inferior mesenteric artery. Large bilateral presumed ovarian mets.
- 2/2/24 – planned for diverting loop colostomy, however due to peritoneal disease at the splenic flexure, this was unable to be performed and an ileostomy was created. Peritoneal cytology positive. Flex sigmoidoscopy and multiple biopsies obtained. Pathology c/w poorly differentiated carcinoma with some features (infiltrative pattern) suggestive of a metastasis but SATB2 stains weakly positive which would be more consistent with a colon primary. CEA 5.7, CA 19.9 - 3,236, CA 125 - 149
- 2/26/24 - began FOLFOX C1 D1 - no 5-FU bolus or leucovorin
- 3/14/24 - presented for C1 D15 and was acutely ill with port infection, muscle stiffness, insomnia, and tremors. Admitted to hospital, port removed. Concern for oxaliplatin hyperexcitability syndrome?

- 3/28/24 - changed therapy to FOLFIRI due to side effects experienced with Oxaliplatin. no 5-FU bolus of leucovorin
- 4/11/24 - CT CAP - after only 1 month of therapy, restaging scans show treatment response with interval decrease long segment sigmoid colon thickening and mild decrease in mesenteric and ovarian metastatic deposits. CEA 7.8, CA 19.9 - 2,101, CA 125 - 27
- 5/16/24 - CT CAP obtained for colorectal surgery shows continued response with decrease in sigmoid colon thickening, decreased size of mesenteric deposits and ovarian metastatic lesions. CEA 1.9, CA 19.9 - 666, CA 125 - 17

Malignant neoplasm of colon (CMS/HCC)

1/25/2024

Initial Diagnosis

Malignant neoplasm of colon (CMS/HCC)

2/26/2024 -
2/28/2024

Chemotherapy

OXALiPlatin (Eloxatin) 107.5 mg in dextrose 5 % 250 mL chemo IVPB, 85 mg/m² = 110 mg, Intravenous, Once, 1 of 6 cycles

Administration: 107.5 mg (2/26/2024)

fluorouracil (Adrucil) 3,050 mg in sodium chloride 0.9 % 100 mL chemo infusion - for home use, 2,400 mg/m² = 3,050 mg, Intravenous, Over 46 hours, 1 of 6 cycles

Administration: 3,050 mg (2/26/2024)

3/28/2024 -
4/13/2024

Chemotherapy

irinotecan (Camptosar) 224 mg in dextrose 5 % 500 mL chemo IVPB, 180 mg/m² = 224 mg, Intravenous, Once, 1 of 1 cycle

Administration: 224 mg (3/28/2024), 224 mg (4/11/2024)

fluorouracil (Adrucil) 3,000 mg in sodium chloride 0.9 % 100 mL chemo infusion - for home use, 2,400 mg/m² = 3,000 mg, Intravenous, Over 46 hours, 1 of 1 cycle

Administration: 3,000 mg (3/28/2024), 3,000 mg (4/11/2024)

4/25/2024 -

Chemotherapy

irinotecan (Camptosar) 224 mg in dextrose 5 % 250 mL chemo IVPB, 180 mg/m² = 224 mg, Intravenous, Once, 4 of 6 cycles

Administration: 224 mg (4/25/2024), 224 mg (5/9/2024), 224 mg (5/23/2024), 224 mg (6/6/2024), 224 mg (6/20/2024), 224 mg (7/5/2024)

panitumumab (Vectibix) 200 mg in sodium chloride 0.9 % 100 mL chemo IVPB, 6 mg/kg = 200 mg, Intravenous, Once, 4 of 6 cycles

Administration: 200 mg (4/25/2024), 200 mg (5/9/2024), 200 mg (5/23/2024), 200 mg (6/6/2024), 200 mg (6/20/2024), 200 mg (7/5/2024)

fluorouracil (Adrucil) 3,000 mg in sodium chloride 0.9 % 100 mL chemo infusion - for home use, 2,400 mg/m² = 3,000 mg, Intravenous, Over 46 hours, 4 of 6 cycles

Administration: 3,000 mg (4/25/2024), 3,000 mg (5/9/2024), 3,000 mg (5/23/2024), 3,000 mg (6/6/2024), 3,000 mg (6/20/2024), 3,000 mg (7/5/2024)

Past medical history:

Past Medical History:

Diagnosis**Date**

- Abdominal pain
- Cancer (CMS/HCC)
colon
- Chronic constipation

Family history:**Family History**

Problem	Relation	Name	Age of Onset
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- Ovarian cysts Sister

Social history:**Social History****Tobacco Use**

- Smoking status: Never
- Smokeless tobacco: Never

Vaping Use

- Vaping status: Never Used

Substance Use Topics

- Alcohol use: Not Currently
- Drug use: Never

Objective

Medications: Reviewed and updated in this Epic encounter

Vitals:

	07/18/24 0845
BP:	94/58
Pulse:	79
Temp:	36.7 °C (98.1 °F)
SpO2:	99%

Physical exam: NAD, NCAT, RRR, MRG, lungs CTAB, abd NTTP, no LE edema, no rashes, bruising. A&O 4/4, normal gait.

Radiology: Images and reports of recent scans 5/16/24 reviewed by me personally with findings as described above in the onc history and below in the assessment.

Laboratory studies: Recent laboratory studies reviewed by me personally with results significant for very mild leukopenia with normal ANC, normal renal and liver function, increasing CEA and CA 19.9.

Assessment and Plan**1. Adenocarcinoma of the sigmoid colon**

- **Stage:** IV - mesenteric/peritoneal mets, bilateral ovarian mets, aorta and IMA encasement
- **Pathologic/genetic features:** MSI low / MMRi. Mutations in ATM, SMAD4, TP53. NRAS/BRAF WT (KRAS copy gain), TMB 1.1 m/MB.
- **Treatment history:** FOLFOX (discontinued after 1 cycle 2/2 poor tolerance/? Oxali hyperexcitability syndrome) | FOLFIRI + Pan 3/28/24 - present

Cancer management:

Ms. Alejandro continues to tolerate FOLFIRI + Panitumumab well and overall has had considerable clinical and radiographic improvement on this regimen compared to time of diagnosis. Unfortunately, while she continues to feel well, her tumor markers have increased today (CA 19.9 666 -> 1386, CEA 2.1 -> 4.7), indicative of biochemical progression.

We are re-checking SigNatera today to better assess her disease burden, though this may be difficult to interpret as her initial SigNatera was checked only about a week after treatment initiation, so a decreased MTM/mL value could still very much be concordant with disease progression. She is also due for repeat CT scans with next cycle. Given her age she has also been referred to genetic counseling and we are checking Ambry germline genetics today.

Ultimately we will need to look at the totality of her clinical picture, including symptoms, imaging, tumor markers, and genetics to determine next steps. If she has an isolated area of disease progression, referral for localized therapy may be an option. However, if she has frank progression at multiple sites we will need to switch systemic therapy. Standard of care options are limited given her oxaliplatin intolerance. With her age and performance status, she would likely be a good trial candidate, so we will look into options. We will also discuss her case at molecular tumor board given her ATM mutation.

Return to clinic in 4 weeks for cycle 5 of FOLFIRI + Pan with CT C/A/P, CMP, CBC, tumor markers.

2. **Encounter for anti-neoplastic therapy** - Fluorouracil 2,400 mg/m² + irinotecan 180 mg/m² + panitumumab 6 mg/kg
3. **AYA Cancer** - Genetic counseling as above. Previously discussed and declined fertility preservation.
4. **Barriers to healthcare** - Transportation, requiring family to bring her to appointments. FAP and Medicaid pending. Financial aid following.

Aaron Pruitt, MD
Fellow, Hematology and Oncology

Patient discussed in detail with attending physician Dr. Reema Patel

Additional Documentation

7/22/24, 11:22 AM

Martinez Alejandro, Rosa (MRN 110710532) DOB: 01/19/1994 Encounter Date: 07/18/2024

Vitals: BP 94/58 Pulse 79 Temp 36.7 °C (98.1 °F) (Oral) Ht 1.499 m (4' 11") Wt 35.4 kg (78 lb 0.7 oz) SpO2 99%
BMI 15.76 kg/m² BSA 1.21 m² Pain Sc 4 (Loc: Ankle)

Flowsheets: Patient Health Questionnaire-2/9, NCCN Distress Thermometer, Interpreter Services

SmartForms: AMB SF STEADI FALL RISK

Communications

No questionnaires available.

Linked Episodes

Oncology Treatment Noted 2/19/2024
Infusion Treatment 1 Noted 3/28/2024

Orders Placed

Labs
Cancer antigen 19-9 (Resulted 7/18/2024, Abnormal)
CEA (Resulted 7/18/2024, Abnormal)
CA 125 (Resulted 7/18/2024)
Cancer antigen 19-9
CBC and differential
CEA
Comprehensive metabolic panel
Magnesium
SigNatera; Yes; No; No; Immediate - Miscellaneous Test

Imaging
CT Abdomen Pelvis w IV Contrast
CT Chest w IV Contrast

Other Orders
Ambulatory referral to Molecular Tumor Board Authorized
Clinic Appointment Request
Infusion Appointment Request
Infusion Appointment Request

Medication Changes

As of 7/18/2024 8:48 AM

None

Medication List at End of Visit

As of 7/18/2024 8:48 AM

	Refills	Start Date	End Date
dexamethasone (Decadron) 4 MG tablet	0	3/28/2024	—
Take 2 tablets (8 mg) by mouth 1 (one) time each day. Take for two days after chemo (on days 2, 3, 16, and 17 of cycle) - Oral			
Notes to Pharmacy: M2CC 3/28			
doxycycline (Adoxa) 100 MG tablet	2	6/20/2024	9/18/2024

	Refills	Start Date	End Date
Take 1 tablet (100 mg) by mouth 2 (two) times a day. Take with a full glass of water and do not lie down for at least 30 minutes after - Oral Notes to Pharmacy: M2CC 6/2024 please			
escitalopram (Lexapro) 10 MG tablet Take 1 tablet (10 mg) by mouth 1 (one) time each day. - Oral Notes to Pharmacy: M2CC 4/25/24	2	4/25/2024	—
hydrocortisone 1 % ointment Use topically as needed twice daily for rash Notes to Pharmacy: M2CC 4/25/24	2	4/25/2024	—
ibuprofen 200 MG tablet Take 2 tablets (400 mg) by mouth every 6 (six) hours if needed for mild pain. - Oral Patient-reported medication	—		—
lidocaine-prilocaine (Emla) 2.5-2.5 % cream Apply as needed, prior to port access Patient not taking: Reported on 4/25/2024 Notes to Pharmacy: M2CC 2/26- label in spanish please	1	2/26/2024	—
loperamide (Imodium A-D) 2 MG tablet 2 mg by mouth every 2 hours as needed for diarrhea; Not to exceed 16 mg in 24 hours Patient not taking: Reported on 7/18/2024 Notes to Pharmacy: M2CC 3/28	5	3/28/2024	—
mirtazapine (Remeron) 7.5 MG tablet Take 1 tablet (7.5 mg) by mouth every night. M2CC please - Oral	2	6/20/2024	—
prochlorperazine (Compazine) 10 MG tablet Take 1 tablet (10 mg) by mouth every 6 (six) hours if needed for nausea or vomiting. - Oral Notes to Pharmacy: M2CC 3/28	5	3/28/2024	—

Visit Diagnoses

Primary: **Malignant neoplasm of sigmoid colon (CMS/HCC) C18.7**
Encounter for antineoplastic chemotherapy Z51.11